

Ethiopian Health Professional
Exit/Licensure Examination
Selected Questions *MSN+PEDIATRIC +SOME*
OF GYNECOLOGY EXIT QUESTIONS

1. A Two -month old infant was brought to a hospital with complaints of cough two days duration. The infant is alert and can feed breast milk. The infant respiration is 52beat/min,pulse rate 110breath/min and temperature is 38°c. The NURSE wants to classify the illness according to IMNCI Guideline. What is the most likely diagnosis of this child?

A) Very severe Pneumonia

B) severe pneumonia

C) Pneumonia

D)Cough or Cold

2.in a patient with a burn injury of 40%TBSA and weight 52kg, The Nurse wants to resuscitate the patient

What will be the minimum calculated ammount of intravenous solution will be administered in the first 8 hour and the type of fluid?

A)2080ml RL

B)4160ml NS

C) 4160ml RL

D)2500ml Dw5%

RL is potassium chloride, and calcium chloride in water. This solution is used to restore blood volume and correct electrolyte

imbalances, particularly in situations like dehydration, burns, and post-surgical or trauma cases.

helps restore fluid balance and combats hypovolemia (low blood volume) caused by burns. Ringer's Lactate is also known as Hartmann's solution and is often the preferred first-line fluid for burn resuscitation.

3. A Nurse observed a medication administration error made by his friend in medical Ward. Due to this the patient developed minor complication and treated.

What will be the first thing the nurse must do?

- A) Confront his friend
- B) Follow secretly and supervises
- C) Report to nurse director
- D) Report to the legal judiciary body

4. A patient refused a life saving treatment. What ethical principle is most relevant in this situation?

- A) Justice
- B) Autonomy
- C) Fidelity
- D) Veracity

5. A 2 years old child presented with fever, vomiting and irritability. A U/A shows that **LEUKOCYTES &NITRITES.**

What is the most likely DX Of this child?

- A) UTI
- B) Gastroenteritis
- C) Meningitis
- D) Pneumonia

LEUKOCYTES &NITRITES.

When white blood cells are present, which can be a sign of infection or inflammation. Nitrites, on the other hand, are a byproduct of bacterial metabolism in the urinary tract, and their presence in urine can also suggest a bacterial infection.

6. A new born baby is brought to hospital for routine check up. The midwife notes that the baby developed yellowish discoloration of the skin and sclera on the 3rd day of life. The baby is breast feeding well and no other symptoms.

What will be the MOST diagnosis of this baby?

- A) Physiological Jaundice
- B) Pathological Jaundice
- C) Hemolytic disease of New born
- D) Breast Feeding jaundice

7. NURSE ABDI WANTS TO ASSES THE ABDOMEN.

WHICH SEQUENCE OF ASSESSMENT IS THE BEST?

- A) Inspection, auscultation, palpation, percussion
- B) Palpation, inspection, auscultation, percussion
- C) inspection, palpation, auscultation, percussion
- D) inspection, auscultation, percussion, palpation

8. A 30 years old Gravida II women comes to health center for ANC follow up. On physical EXAMINATION she is 32 weeks gestational age FHB is positive. PR=88BPM

On laboratory investigation urine analysis is:- Protein +ve, VDRL=+ve and HGB=9g/dl.

Based on this SCENARIO what is her DIAGNOSIS ?

- A) Anemia and pre-Eclampsia
- B) Anemia and syphilis
- C) Syphilis and pre eclampsia
- D) Eclampsia and syphilis

the most likely diagnosis is preeclampsia with superimposed preeclampsia and syphilis

Explanation:

Preeclampsia, a serious complication of pregnancy, is often characterized by high blood pressure, protein in the urine, and potentially other symptoms like swelling, headache, and vision changes. In this case, the proteinuria and timing of the diagnosis suggest it's a concern, possibly superimposed by the infection of syphilis.

Syphilis, a bacterial infection, can be transmitted during pregnancy and has serious consequences for both mother and baby. The positive VDRL test confirms this infection, requiring immediate treatment to prevent complications.

In conclusion: The combination of preeclampsia-related findings (proteinuria) and the positive VDRL test for syphilis indicates a complex situation requiring prompt medical attention.

9 A 30 years old pregnant women comes to OBY GYN WARD with Complaints of bright red Vaginal bleeding. On EXAMINATION The lie is transverse and also No pain during palpation.

What is the most causes of this BLEEDING ?

- A) Placenta abruption
- B) Placenta Previa
- C) Local lesion
- D) Abortion

9. A 45 years old anemic woman came to emergency department with

complaint of headache, featgue and edema. After examination, it is confirmed

that 80%of her kidney's failed with the serum creatinine level of 1.8mg/dl (normal

range 0.6 -1.2 mg/dl).

What is the most likely pathophysiological stage of the patient?

A. Renal insufficiency

B. Reduced renal reserve

C. End stage renal failure

D. Retention of nitrogenous wastes

- E) **Renal insufficiency**: refers to a condition where the kidneys are unable to adequately filter waste products from the blood, leading to a buildup of waste and fluid.
- F) The patient's serum creatinine level is elevated (1.8 mg/dL), which is a key indicator of impaired kidney function. Normal creatinine levels are 0.6-1.2 mg/dL, and elevated creatinine levels signify a build-up of waste in the bloodstream, indicating impaired kidney function.
- G) The patient's symptoms of headache, fatigue, and oedema are consistent with the pathophysiological processes of renal insufficiency.
- H) **End-stage renal failure**: typically involves a more severe level of kidney function impairment than that described in the question.
- I) **Reduced renal reserve**: refers to a decreased capacity of the kidneys to handle increased demands. While this may be present, it's less likely to be the primary cause of the patient's symptoms.
- J) **Retention of nitrogenous wastes**: is a symptom of renal insufficiency and is a consequence of the kidneys failing to eliminate nitrogenous wastes. The question asks for the most likely pathophysiological stage, not the immediate consequence of the condition.

10.A26 years old male patient was admitted to an ICU and put on mechanical ventilator following car accident .on assessment his eye are not open to any stimuli ,produces incomprehensible sounds and has abnormal flexion of motor response .

What is the level of consciousness of this patient using Glasgow coma scale?

- A, Semiconsciousness
- B, Somnolent

C, Stupors

D, Comatose



HOW TO SOLVE



Assign GCS scores for each aspect of responsiveness: eye opening, verbal response, and motor response, and then sum the scores.

1. 1. Step 1

Assign GCS score for eye opening

- The patient does not open his eyes to any stimuli, so the eye opening score is 1.

2. 2. Step 2

Assign GCS score for verbal response

- The patient produces incomprehensible sounds, so the verbal response score is 2.

1. 4. Step 4

Calculate the total GCS score

- The GCS score is the sum of the individual scores: 1 (eye) + 2 (verbal) + 4 (motor) = 7.

Solution

The Glasgow Coma Scale score for this patient is 7

11. A 29-year-old female patient came to adult OPD with complaint of nuchal

rigidity, vomiting and seizure. Physical examination findings indicated positive

Kernig sign and Brudzinkski's sign and petechial spread over the body. Laboratory

findings show that CSF appears cloudy, decreased glucose concentration,

elevated protein levels and elevated WBC and RBC counts. What is the most likely

medical diagnosis for this patient?

A. Meningococcal encephalitis

B. Meningococcal meningitis

C. Viral meningitis

D. Viral encephalitis

Meningoencephalitis:

If both the meninges and brain are infected, the condition is called meningoencephalitis.

Diagnostic Evaluation

By +ve signs

Clinical manifestation Confirmative sign:

Positive kerning's sign

Brudziniski's Sign

Physical examination

History collection

CSF evaluation for pressure, proteins, glucose and leukocytes.

Blood test

CBC

Blood culture

MRI

CT scan

12.A 40 years old HIV positive woman came to ART Clinic with complaint of

intense itching on physical examination the patient had firm papule with variable

stage of development on her trunk and face. What is the most likely explanation

for this finding?

A, Cellulitis

B, Prurigo

C, Herpes simplex

D, Molluscum contagiousum

Cellulitis, Prurigo, Herpes simplex, and Molluscum contagiousum are all skin conditions, but they have different causes, symptoms, and treatments. Cellulitis is a bacterial skin infection, while Prurigo and Herpes simplex are viral infections and Molluscum contagiousum is a poxvirus infection.

CELLULITIS

- **Symptoms: Intense itching and the formation of nodules.**

Herpes Simplex

- **Cause: Viral infection, typically caused by [HSV-1](#) (oral herpes) or [HSV-2](#) (genital herpes).**
- **Symptoms: Itching, tingling, followed by blisters that can be painful.**
- **Symptoms: Intense itching and the formation of nodules.**

13. A 60 year old man who had been smoking a pack of cigarettes daily for the last twenty years presented with one year history of increasing dyspnea on exertion and accumulation dry cough. Chest examination reveals hyper resonance all over the lungs and decreased breath sounds. Which of the following is most likely diagnosis?

- A) Bronchiectasis
- A) Asthma
- C) Emphysema
- D, chronic bronchitis

What is bronchiectasis?

[Bronchiectasis](#) is a progressive lung condition caused by damage to bronchi (large air passages) in the lungs. With bronchiectasis, the walls of the bronchi become thickened from ongoing inflammation or infection.

People with this condition cough up large amounts of mucus, especially during flareups. Flareups of bronchiectasis are referred to as exacerbations. [During an exacerbation](#), you will also find it harder to breathe.

Bronchi are designed to enable free breathing by letting air enter the lungs. Bronchiectasis occurs when the bronchi in the lungs become chronically inflamed and thickened. Over time, the thickening of the bronchial walls and subsequent scarring make it hard to move mucus out of the lungs. Recurring infections also become more likely.

Bronchiectasis involves damage to the large airways and is often linked to infections or genetic conditions, while

emphysema is a form of COPD primarily caused by smoking and leads to damage to the alveoli, the small air sacs in the lungs.

14. which one of the following signs is associated with meningitis?

- A. Rousing's sign
- B. kernig's sign
- C. Tinel's sign
- D. Trousseau's sign

14. A 52 year-old female presents to the clinic with four months of fatigue, lower extremity burning pain and numbness. Laboratory test reveals Hgb=8.5g/d, MCV=118fl, WBC=4000/ μ l and platlet=150,000/ μ l. What is the most diagnosis?

- A. Aplastic anemia(normocytic)
- B. Folate- deficiency anemia
- C. iron-deficiency anemia
- D. vitamin B12 deficiency anemia

The mean corpuscular volume (MCV) measures the average size of the red blood cells. The average MCV ranges from 80 to 100 femtoliters.

MCV can be high, low, or normal:

- **Microcytic (micro=small): MCV level less than 80 fL**
- **Normocytic: MCV level between 80 and 100 fL**
- **Macrocytic (Micro=large): MCV level greater 100 fL**
- **Macrocytic anemia (high MCV) is most often due to decreased levels of vitamin B12 or folate in the body. Normally, red blood cells are shaped as biconcave discs, like the shape you get when you squeeze a marshmallow. B12 and folate are important building blocks of the RBC wall membrane. If their levels are decreased, there are fewer materials available to build those membranes, and it turns out that the surface area and the amount of membrane to contain the inside of the RBC is less when the cell is more round instead of concave. That makes the RBC appear larger, and that leads to a higher MCV.**
- **High MCV in itself will not cause any symptoms. It can be associated with anemia if the red blood cell levels decrease and the oxygen delivery to tissues is decreased. If the elevated MCV is due to vitamin B12 deficiency, there can be symptoms associated with pernicious anemia (the term to describe low B12).**

APLASTIC ANEMIA

Aplastic anemia is a condition that happens when your bone marrow stops making enough new blood cells. The condition leaves you tired and more prone to infections and uncontrolled bleeding.

- **Unexplained or easy bruising.**
- **Nosebleeds and bleeding gums.**
- **Prolonged bleeding from cuts.**
- **Pale skin.**
- **Skin rash.**
- **Fever.**

■ Thanks to All

-PEACE BE UP ON YOU 

HAVE A NICE TIME 